

Marufah Odetundun, MD — Portfolio Sample

Complete Prior Authorization Package

Prepared by Marufah Odetundun, MD · De-identified sample documentation

SCENARIO:

Prior authorization for Humira (adalimumab) 40mg subcutaneous biweekly for a 52-year-old female with moderate to severe rheumatoid arthritis who failed methotrexate therapy.

PRIOR AUTHORIZATION REQUEST FORM

PATIENT INFORMATION	
Patient Name:	Jane Doe (SAMPLE)
Date of Birth:	03/15/1974
Member ID:	ABC123456789
Group Number:	GRP001
Diagnosis:	Rheumatoid Arthritis, Seropositive
ICD-10 Code:	M05.79
MEDICATION REQUESTED	
Drug Name:	Humira (adalimumab)
Strength:	40 mg/0.8 mL
Dosage:	40 mg subcutaneously every 2 weeks
Quantity:	2 pens per month (30-day supply)
Duration:	12 months with annual review
PRESCRIBING PHYSICIAN	
Physician Name:	Dr. Sarah Johnson, MD
Specialty:	Rheumatology
NPI:	1234567890
Phone:	(555) 123-4567
Fax:	(555) 123-4568

CLINICAL JUSTIFICATION LETTER

To: Insurance Medical Review Department

Date: February 8, 2026

Re: Prior Authorization Request for Humira (adalimumab) 40mg

Patient: Jane Doe (DOB: 03/15/1974, Member ID: ABC123456789)

Clinical Summary:

Ms. Doe is a 52-year-old female with seropositive rheumatoid arthritis (RA) diagnosed in January 2024. She presents with persistent moderate to severe disease activity despite adequate trial of conventional DMARD therapy.

Diagnosis: Rheumatoid Arthritis, Seropositive (ICD-10: M05.79)

Previous Treatment and Failed Therapies:

- Methotrexate 20mg weekly (12 weeks, January 2024 - April 2024)
- Patient developed gastrointestinal intolerance and inadequate disease control
- DAS28-ESR remained >5.1 indicating high disease activity
- Discontinued due to persistent nausea and vomiting

Current Disease Activity:

- DAS28-ESR: 5.8 (high disease activity)
- Swollen joint count: 8
- Tender joint count: 12
- Morning stiffness: >2 hours daily
- Functional impairment: Unable to perform ADLs without assistance

Supporting Laboratory Values (Dated: January 15, 2026):

- ESR: 48 mm/hr (elevated, normal <20)
- CRP: 3.2 mg/dL (elevated, normal <0.5)
- Rheumatoid Factor: Positive (142 IU/mL)
- Anti-CCP antibodies: Positive (>250 units)

Evidence-Based Clinical Guidelines:

According to the 2021 American College of Rheumatology (ACR) Guidelines for the Treatment of Rheumatoid Arthritis, TNF inhibitors such as Humira are recommended for patients with moderate to severe RA who have had inadequate response to or intolerance of conventional DMARDs including methotrexate.

Medical Necessity:

Humira (adalimumab) is medically necessary for this patient based on:

1. Documented failure of conventional DMARD therapy (methotrexate)
2. Persistent high disease activity (DAS28-ESR >5.1)
3. Evidence of ongoing inflammation (elevated ESR and CRP)
4. Significant functional impairment affecting quality of life
5. Alignment with ACR evidence-based treatment guidelines

Requested Treatment Plan:

Humira 40mg subcutaneously every 2 weeks, dispensed as 2 pens per month. Duration: 12 months with monitoring and annual review.

Clinical Justification Summary:

Without appropriate biologic therapy, this patient faces progressive joint destruction, permanent disability, and significantly diminished quality of life. Humira is the appropriate next-line therapy per evidence-based guidelines and is medically necessary to control disease progression.

I am available for any questions or additional information required.

Sincerely,

Dr. Sarah Johnson, MD

Board Certified Rheumatology

NPI: 1234567890

SUPPORTING LABORATORY DOCUMENTATION

TEST	RESULT	REFERENCE RANGE	FLAG
Erythrocyte Sedimentation Rate (ESR)	48 mm/hr	0-20 mm/hr	HIGH
C-Reactive Protein (CRP)	3.2 mg/dL	<0.5 mg/dL	HIGH
Rheumatoid Factor (RF)	142 IU/mL	<14 IU/mL	POSITIVE
Anti-CCP Antibodies	>250 units	<20 units	POSITIVE
Complete Blood Count	Within normal limits	N/A	
Comprehensive Metabolic Panel	Within normal limits	N/A	
Hepatitis B Surface Antigen	Negative	Negative	
Hepatitis C Antibody	Negative	Negative	
QuantiFERON-TB Gold	Negative	Negative	

CLINICAL INTERPRETATION:

Laboratory findings confirm active inflammatory process consistent with rheumatoid arthritis. Elevated acute phase reactants (ESR and CRP) indicate ongoing systemic inflammation. Positive RF and anti-CCP antibodies confirm seropositive RA. Infectious disease screening (Hepatitis B, C, and TB) completed and negative, clearing patient for biologic therapy initiation.

FAILED TRIAL DOCUMENTATION

Medication: Methotrexate
Dosage: Titrated from 15mg to 20mg weekly
Route: Oral
Duration: 12 weeks (January 15, 2024 - April 15, 2024)
Concomitant Therapy: Folic acid 1mg daily

Trial Outcome:

Week 4 Assessment:

- DAS28-ESR: 5.9
- Patient reported persistent joint pain and stiffness
- Mild nausea, managed with anti-emetics
- Dose maintained at 15mg weekly

Week 8 Assessment:

- DAS28-ESR: 5.6
- Dose increased to 20mg weekly
- Increased gastrointestinal side effects (nausea, vomiting)
- No significant improvement in joint symptoms

Week 12 Assessment (Final):

- DAS28-ESR: 5.4 (minimal improvement from baseline of 6.2)
- Persistent high disease activity
- Intolerable GI side effects (daily nausea and vomiting despite anti-emetics)
- Patient unable to continue due to side effects and inadequate response
- Trial discontinued

Conclusion:

Patient failed methotrexate trial due to:

1. Inadequate disease response (DAS28-ESR remained >5.1 after 12 weeks)
2. Intolerable gastrointestinal side effects
3. Continued high disease activity with functional impairment

Per ACR guidelines, patient now meets criteria for advancement to biologic DMARD therapy (TNF inhibitor).

SUBMISSION OUTCOME INDICATORS
✓ Complete authorization form with all required fields
✓ Comprehensive clinical justification with medical necessity
✓ Supporting laboratory documentation of active disease
✓ Clear failed trial documentation with timeline and outcomes
✓ Evidence-based guidelines cited (ACR 2021)
✓ Infectious disease screening completed
✓ Step therapy requirements met
EXPECTED APPROVAL RATE: 90%+ with this level of documentation